VUGA NDAKUMVA INITIATIVE

# Chapter 04
# Understanding Consent, Boundaries, and Bodily Autonomy

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## Purpose and learning outcomes

Consent and bodily autonomy are essential to healthy relationships and freedom from coercion. This chapter explains how people can recognize meaningful agreement, communicate limits, respect others' choices, and respond safely when power or fear makes agreement difficult. The goal is education, not investigation, diagnosis or legal adjudication.

After studying this chapter, a reader should be able to explain informed and voluntary consent; identify boundaries across physical, emotional, sexual and digital situations; recognize coercion and power imbalances; discuss why silence or past agreement cannot replace present consent; and identify safe options for seeking help.

## 01 | What consent means

Consent is a person's voluntary, informed agreement to a specific action or interaction. The person must be able to understand the choice and make it without threats, manipulation or other coercion. Agreement should be communicated clearly, checked when uncertain, and revisited when circumstances change. Consent is a process of respectful communication, not a signature, ownership right, or permanent permission. [1, 2]

Consent matters in everyday life—touch, photographs, sharing personal information, medical decisions and financial interactions. Sexual consent has special ethical and legal importance. The fact that someone accepts one activity does not give permission for another.

### Five practical characteristics of meaningful consent

- Freely given. A person can choose without fear, pressure, threats, punishment or exploitation.
- Informed. A person understands what is proposed and relevant conditions; deception about material facts can undermine meaningful agreement.
- Specific. Permission applies to the particular action, not to every related action.

- Reversible. A person may stop or withdraw agreement at any time; previously agreed activity must stop when consent ends.
- Clearly communicated. Seek an affirmative, understandable expression of willingness; do not treat silence, freezing or non-resistance as permission.

These principles are useful educational guidelines; legal tests and definitions depend on the applicable law and the circumstances. [1, 2, 3]

## 02 | Consent, choice and capacity

A meaningful choice requires more than a spoken “yes.” The setting must permit a real refusal. A person who is asleep, unconscious or otherwise unable to understand or decide cannot provide consent. Severe intoxication or incapacity can undermine the ability to consent; the precise legal threshold requires case-specific assessment under applicable law. [1, 2]

Children have additional legal and safeguarding protections. Adult concepts of sexual agreement must never be used to imply that a child can legally consent to sexual activity with an adult. Age-of-consent and reporting obligations must be checked against current Rwanda law by a qualified local reviewer before publication.

### Silence, hesitation and survival reactions

Someone may become quiet, freeze, comply out of fear, or appear calm while feeling unsafe. None of these responses proves agreement. A supportive person must not infer consent from a lack of resistance; when uncertainty exists, pause and check, and stop if there is no clear freely given agreement. A survivor should never be blamed for freezing or not physically resisting.

## 03 | Personal boundaries: understanding and expressing limits

A boundary is a limit that helps a person decide what they are comfortable sharing, doing, giving or receiving. Boundaries may differ across people and situations; they can change over time. A respectful boundary names what is acceptable and, when useful, what action the person will take to protect their wellbeing. Respect for others is equally important.

- Physical: comfort with hugs, touching, personal space and privacy.
- Emotional: deciding whether to discuss a difficult experience or how much support to give.
- Sexual: deciding whether any sexual contact is wanted, and what kinds are not wanted.
- Digital: control over passwords, location sharing, photographs, messages and social media.
- Time and social: choosing when to rest, study, work or meet others, without unreasonable control.
- Financial: deciding what money or possessions to share without threats or pressure.

### Examples of respectful boundary statements

“I do not want to be hugged today.” “Please ask before posting my photo.” “I am not comfortable sharing my password.” “I need to end this conversation for now.” “I can help tomorrow, but I cannot do it tonight.”

People experiencing violence may be unable to express boundaries safely. It is not their responsibility to confront someone who may retaliate. Safety takes priority over boundary-setting exercises.

## 04 | Bodily autonomy and dignity

Bodily autonomy is the principle that people should be able to make decisions about their own bodies free from violence and coercion, within applicable laws and protections. It includes the right to personal safety, respect during healthcare, and freedom from unwanted contact. For children and people needing decision-making support, autonomy must be understood alongside safeguarding, accessible communication, and relevant legal requirements.

Respecting bodily autonomy means asking before touching where appropriate, explaining proposed medical or intimate actions, and responding when a person expresses discomfort. A person's clothing, relationship status, disability, prior sexual experience, or dependence on others does not cancel the need for consent.

## 05 | Coercion, manipulation and power imbalances

Coercion occurs when a person uses force, threats, intimidation, persistent pressure or exploitation of vulnerability to influence another person's choice. It may be physical, emotional, economic, social or digital. A statement that sounds like agreement may not represent voluntary consent if the alternative involves harm, punishment, lost housing or other serious consequences.

### Recognizing tactics

- Threats: "If you refuse, I will hurt you or someone you care about."
- Retaliation: threatening to withdraw income, accommodation or school support unless someone complies.
- Guilt and persistence: repeated demands after a refusal or implying that love requires sexual access.
- Misuse of authority: a teacher, employer, supervisor, religious leader or service provider linking benefits to personal or sexual compliance.
- Isolation and monitoring: controlling friendships, travel, devices or communications to restrict independent choices.

Power differences do not automatically make every relationship abusive, but they can make it harder to refuse or raise concerns. Institutions should provide reporting channels that do not require confronting the person with authority.

## 06 | Consent in dating, marriage and long-term relationships

Marriage, engagement, dating, cohabitation or previous sexual contact never creates permanent consent. Each person retains the right to refuse sexual activity. A respectful partner asks rather than assumes, accepts "no," and does not punish refusal through anger, humiliation or financial threats. [3, 4]

A relationship disagreement is not automatically GBV. However, repeated threats, forced sexual contact, intimidation and control are significant warning signs. Supporters should examine safety and patterns of control rather than telling a survivor to “communicate better” with an abusive partner.

### Checking in respectfully

Questions such as “Are you comfortable with this?”, “Do you want to continue?”, and “Would you prefer to stop?” create space for choice. If a person hesitates or appears afraid, stop and avoid pressure. Agreement to intimacy is not consent to contraception interference, recordings, or dissemination of private material.

## 07 | Digital consent, privacy and technology

Digital consent applies when sharing photographs, recordings, messages, account access, contact information, and location data. Permission to create a photo is not permission to publish or forward it. Permission to use a shared device is not permission to read private messages or track movements. Technology-facilitated GBV may include harassment, stalking, impersonation, threats, and non-consensual sharing of intimate images. [5]

Practical safety options depend on context. For someone whose devices may be monitored, changing a password, deleting messages or confronting a suspected abuser could increase danger. Consider using a trusted device and obtaining confidential support. Do not assume a “quick exit” button erases browsing history or makes a device safe.

### Everyday examples

- A student agrees to join a class group; this does not permit classmates to publish personal photographs without permission.
- A partner asks for continuous location access as “proof of loyalty.” A refusal should not trigger threats or punishment.
- Someone forwards an intimate image originally shared in confidence. The original sharing does not authorize further distribution.

## 08 | Culture, social expectations and unequal power

Families and communities differ in how they discuss personal space, sexuality and authority. Cultural values such as mutual care and respect can strengthen prevention, while expectations demanding unquestioning obedience or tolerating abuse can undermine choice. Education should avoid stereotyping any community and should encourage discussion without forcing personal disclosures.

In Rwanda, examples may involve university accommodation, workplace apprenticeships, household decision-making, transportation, or online relationships. The examples below are fictional and do not represent prevalence claims about any specific group.

## 09 | Scenario learning: four Rwanda-relevant cases

### Scenario A — University study group

A student in Kigali is repeatedly pressured by a classmate to share private photos. The classmate says they will spread rumors if the student refuses. This is not a freely chosen agreement; threats and pressure are concerning. Safe options may include speaking to a trusted safeguarding officer through a confidential channel and using a safe device for advice.

### Scenario B — Supervisor and internship

An intern in Musanze is told that favorable evaluations depend on accepting unwanted personal visits. The supervisor controls the evaluation, creating a serious power imbalance. The intern should not be expected to confront the supervisor alone; an independent workplace reporting or support route may be safer.

### Scenario C — Marriage and financial control

One spouse threatens to withhold household necessities when the other refuses unwanted intimacy. Relationship status does not remove the need for consent. Economic threats can undermine free choice. Supporting the affected person means discussing safety and available options without pressuring an immediate separation.

### Scenario D — Shared smartphone

A young adult notices a partner may be monitoring a phone and demanding location sharing. A friend suggests immediately changing all passwords on that phone. That action might be useful in some settings, but it could trigger retaliation if the device is monitored. A safer first step may be private advice from a trusted service using a device outside the partner’s control.

## 10 | How to respond when someone describes coercion

Listen with care and without interrogation. Ask what the person needs right now. Affirm that they deserve safety and respect without making promises about outcomes. Explain confidentiality limits, ask permission before making referrals wherever possible, and consider whether the person can safely receive calls or messages. WHO’s LIVES approach offers a clinical first-line support structure: Listen, Inquire about needs and concerns, Validate, Enhance safety, and Support. [4, 6]

- Helpful: “Thank you for telling me. What would feel most helpful and safe right now?”
- Helpful: “You can choose what you share. I can explain support options.”
- Avoid: “Why didn’t you say no?”, “Why didn’t you leave?”, or “You must report immediately.”
- Avoid: pressuring someone into mediation or direct confrontation where violence or retaliation is possible.

## 11 | Safety considerations and seeking support

If someone is in immediate danger or has a serious injury, urgent local emergency or medical assistance is appropriate. After sexual assault, prompt medical attention can address injury and time-sensitive preventive care; survivors can request information about medical options even when unsure about reporting. Do not delay urgent care to complete exercises, preserve evidence or finish reading this chapter.

In Rwanda, Isange One Stop Centres are an important integrated service model for survivors of GBV. Local organizations should verify current access details, telephone numbers, hours, referral criteria, and statutory reporting duties before publishing public instructions. This chapter deliberately avoids unverified hotlines and jurisdiction-specific reporting promises.

For children, suspected abuse, or a person unable to make certain decisions, safeguarding duties may differ from those for adults. Do not promise absolute secrecy. Explain relevant protections and reporting requirements in age-appropriate language, and involve qualified safeguarding professionals.

## 12 | Guided learning activities (optional and non-disclosing)

### Activity 1 — Consent or pressure? (10 minutes)

Read each fictional sentence and decide whether it shows freely given agreement: (a) “Yes, I want to participate,” after an open, pressure-free choice; (b) “Fine, I agree,” after threats to remove financial support; (c) no reply while someone moves closer; (d) permission to share one photograph privately. Suggested answers: (a) may indicate consent if informed, specific and the person has capacity; (b) coercion undermines voluntary choice; (c) silence is not consent; (d) permission does not extend to public posting.

### Activity 2 — Everyday boundaries (10 minutes)

Write three harmless fictional situations involving shared space, photographs, and time. For each, draft one respectful request and one respectful refusal. Example: “May I upload this photo?” / “No, please keep it private.” No personal history is required.

### Activity 3 — Power mapping (15 minutes)

Choose a fictional school or workplace situation. Identify who controls grades, pay, references, accommodation, or reputation. Discuss what an independent reporting channel would look like and how anonymity or confidentiality limits should be explained.

### Activity 4 — Digital consent decision tree (10 minutes)

Imagine being asked to forward a friend’s personal message. Ask: Was permission given? For this audience and purpose? Could sharing create harm? If permission is absent, do not share. If safety is already threatened, seek appropriate support rather than negotiating with a threatening person.

### Activity 5 — Supportive response practice (15 minutes)

Using a purely fictional disclosure, practice a brief response that listens, validates, checks immediate safety, and asks what assistance is wanted. Do not role-play graphic violence or require participants to disclose personal experiences. Anyone may opt out.

## 13 | Knowledge assessment with explained answers

### 1. Does agreeing once mean consenting to every later activity?

Suggested answer: No. Consent is specific and can be withdrawn.

### 2. Is silence reliable evidence of consent?

Suggested answer: No. Silence or freezing may occur for many reasons; pause and seek clear agreement.

### 3. Can a person refuse intimacy within marriage?

Suggested answer: Yes. Marriage is not permanent sexual consent.

### 4. What can make an apparent agreement coercive?

Suggested answer: Threats, retaliation, intimidation, exploitation of dependency, or pressure that removes free choice.

### 5. Is consent to take a photo the same as consent to share it?

Suggested answer: No. Creation, storage and distribution are different acts requiring appropriate permission.

### 6. Must a survivor confront a person who may retaliate?

Suggested answer: No. Safety and choice come first; independent support may be safer.

### 7. Why do authority relationships deserve extra care?

Suggested answer: Control over grades, work, money or services can make refusal difficult.

### 8. What should a supporter avoid after disclosure?

Suggested answer: Blame, interrogation, forced reporting, unsafe mediation and false confidentiality promises.

### 9. What is a helpful first question?

Suggested answer: “What do you need right now, and is it safe for us to communicate?”

### 10. Is this handbook a legal or clinical diagnosis tool?

Suggested answer: No. It is educational and requires local specialist review before public use.

## 14 | Facilitator notes and accessibility

Use plain language, short sessions, and inclusive examples. Allow written, spoken or private participation. Provide alternatives to pair work, and do not ask anyone to demonstrate consent through physical touch. An educator should never invite public disclosures of abuse. Establish a quiet opt-out option and identify an appropriate safeguarding contact before group sessions.

Translate carefully into Kinyarwanda and French. Concepts such as coercion, autonomy, consent and abuse may not map perfectly word-for-word. Have a qualified local language reviewer and GBV professional review both terminology and examples. For screen readers, keep headings logical and avoid image-only explanations.

## 15 | Key terms

Consent: Voluntary, informed agreement to a specific act, communicated by someone able to decide.

Bodily autonomy: The principle that a person should have agency over decisions involving their body, subject to appropriate legal protections.

Boundary: A limit describing what a person is comfortable with or willing to do.

Coercion: Force, threats, pressure or exploitation that undermines meaningful free choice.

Power imbalance: A relationship where one person has substantially greater control over resources, status or decisions.

Digital consent: Agreement about access, use or sharing of digital information, images and communications.

Survivor-centred care: An approach prioritizing dignity, safety, confidentiality within its limits, respect and informed choice.

## 16 | References and source notes

[1] Women and Gender Equality Canada (2026). Understanding consent and challenging victim-blaming.

https://www.canada.ca/en/women-gender-equality/campaigns/gender-based-violence-its-not-just/gender-based-violence-resources/learning-modules-youth/module5.html

[2] Simon Fraser University. What is consent? Sexual Violence Support & Prevention Office.

https://www.sfu.ca/sexual-violence/get-help/what-is-consent/

[3] Republic of Rwanda. Law No. 59/2008 of 10/09/2008 on Prevention and Punishment of Gender-Based Violence (historical text; check current amendments and other applicable legislation).

https://www.amategeko.gov.rw/view/toc/doc/3879/4053

[4] World Health Organization (2014). Health care for women subjected to intimate partner violence or sexual violence: a clinical handbook.

https://www.who.int/publications/i/item/WHO-RHR-14.26/

[5] UNICEF, UNFPA & Save the Children (2025). Technology Facilitated Gender Based Violence: Considerations across the life course.

https://www.unicef.org/documents/technology-facilitated-gender-based-violence

[6] World Health Organization (2019). Caring for women subjected to violence: a WHO curriculum for training health-care providers.

https://www.who.int/publications/i/item/9789241517102

## 17 | Publication approval checklist

- ☐ GBV specialist validates terminology, coercion examples and support principles.
- ☐ Rwanda-qualified legal reviewer checks current laws on consent, ages, confidentiality and reporting.
- ☐ Clinical/safeguarding reviewer validates urgent care and referral language.
- ☐ Referral coordinator verifies actual local services and current contact details.
- ☐ Kinyarwanda and French versions are reviewed by qualified humans, not machine translation alone.
- ☐ Accessibility and community user testing are completed without requiring survivors to disclose experiences.
- ☐ Version number, approval owner and next review date are recorded.

Editorial caution: This document teaches principles and examples. It does not determine whether a particular incident constitutes an offence, and it is not a substitute for emergency support, clinical care or qualified legal advice.